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Nicholas Scarr <nscarr@quantlogic.com>

Qlaim 1816 N Scarr - Fw: Our Ref: 90964727158, Insurer Ref: LBGSBP0073077, Policyholder: Mrs Louisa Scarr, Mr Nicholas Scarr SO24 9BE @909647271:58 #Adjuster#Correspondence#

1 message

Nicholas Scarr <nscarr@quantlogic.com> 19 August 2025 at 14:56
To: Andy Lyne <Andy.Lyne@qlaims.com>
Cc: Sarah Romain <sarah.romain@qlaims.com>, Louisa Scarr <scarrlouisa@gmail.com>, Claims <Claims@qlaims.com>
Bcc: Jane Coppard <JaneCoppard@outlook.com>, Malcolm Harvey <mharvey@qlaims.com>

Dear Andy,

This correspondence sets out our unequivocal position regarding the health and environmental risks posed by the sewage contamination incident at our property, and the consequent requirement for the permanent removal of contaminated materials.

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1. Medical Vulnerability of Mrs Louisa Scarr

Mrs Louisa Scarr's medical history places her in one of the highest clinical risk categories for microbial exposure. Her record includes:

- Clostridioides difficile (C. difficile) – a spore-forming, toxin-producing bacterium capable of persisting in the environment and resistant to standard disinfectants;
- Klebsiella pneumoniae (NDM strain) – a carbapenem-resistant "superbug" designated a critical priority by the World Health Organization due to limited treatment options and high mortality risk;
- Sepsis – an episode so severe in 2013 that critical care intervention and the administration of last rites were required.

These infections severely compromised Mrs Scarr's gastrointestinal and systemic immune defences. The risk of recurrence remains significant, especially with environmental exposure to spores and resistant organisms.

The attached medical letter from Dr. Van Gant underscores this vulnerability. He confirms that the only safe standard for Mrs Scarr's return to the property is one equivalent to "hospital clean". Nothing short of this would protect her from foreseeable microbial harm.

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2. Timeline and Environmental Conditions

- Initial Exposure Period: Raw sewage remained on the ground floor for approximately 20 weeks. Beyond an initial superficial clean, the contamination was untreated and pooled under floorboards until Bio Response began professional drying and sanitation.
- Rat Infestation: The prolonged presence of sewage and organic waste caused a severe rat infestation, described by our pest control specialist (an industry veteran) as one of the worst he has encountered.
- Bio Response Findings: Bio Response advised unequivocally that the kitchen units were compromised and required full removal. They also identified widespread microbial and moisture damage throughout both floors, including contamination of household furnishings and electronics such as the Everhot cooker and t televisions.
- Strip-Out Observations: When strip-out began more than three months after the incident, there was still visible dampness, sewage odours, and residual pooling beneath surface structures.
- Upstairs Contamination Indicators: Moisture cracking, persistent odours, and dampness on the first floor suggest airborne microbial migration and humidity-driven contamination.
- Sanitisation Limitations: Despite disinfectant application, ATP testing at the site of the former Everhot kitchen unit recorded a reading of 27+, well above safe thresholds and confirming active biological contamination.
- Drying and Scrubbing Measures: While industrial drying machines and air scrubbers are deployed, they cannot reverse the prolonged contamination or render porous materials safe.

Airborne Pathogen Risk: Industry authorities are explicit on this point:

- The CDC warns that sewage releases generate bioaerosols capable of travelling well beyond the immediate contamination site.
- The EPA notes that sewage-related pathogens may remain suspended in indoor air, creating inhalation risks long after the initial incident.
- The IICRC S500 and S520 standards confirm that porous and semi-porous building materials impacted by sewage cannot reliably be sanitised to safe levels.

Taken together, these findings explain why the property has remained uninhabitable and microbially unstable for nearly five months, with the risk of airborne dissemination across both floors.

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3. Rejection of Sampling as a Viable Approach

At the outset, Bio Response advised clearly that effective decontamination of porous materials exposed to sewage could not be guaranteed. Their advice was that:

- Pathogens—particularly spores and resistant bacteria—would remain embedded in woodwork, insulation, kitchen units, and soft furnishings;
- Surface swab testing would be neither scientifically conclusive nor commercially viable;
- Two small swabs per item (the apparent industry norm) would be wholly inadequate given the scale of contamination;
- Even comprehensive microbial testing would exceed the value of affected items and still fail to offer 100% safety assurance.

If any later suggestion from Bio Response to insurers contradicts this, it would stand at odds with the clear in-person guidance we received during assessment. Their position at the time was unambiguous: kitchen units, sewage-exposed furnishings, and electronics were not recoverable and should be removed.

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4. Justified Material Removal

Following both expert advice and recognised international remediation protocols (CDC, EPA, IICRC, UK HSE), the following items must be removed:

- All kitchen units, due to sewage contact, structural compromise, and microbial absorption;

- All soft furnishings, including mattresses, carpets, upholstered furniture, and all fabric-based materials, both on the ground and first floors. Their porous nature and exposure make them impossible to render safe;

- All affected electronics, where exposure to moisture and airborne contaminants renders them unsafe.

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5. Summary Position

Our position is grounded in both medical necessity and environmental science. We reaffirm that:

- Porous, sewage-exposed materials cannot be rendered safe by sanitisation;
- Airborne pathogen risks extend contamination beyond visible areas;
- For a healthy household, this exposure would still be unacceptable—yet for Mrs Scarr, it is clinically dangerous;
- As confirmed by Dr. Van Gant, nothing less than hospital-grade cleanliness will make the property safe for reoccupation;
- Retaining contaminated materials, even after disinfection or speculative testing, presents a foreseeable and unacceptable risk to health.

In view of the very long delays already endured, we respectfully request that authorization for the removal of the contaminated materials be sanctioned within the next seven days.

Accordingly, the removal of the specified items is not optional but medically and environmentally essential. We ask that this position, together with the attached letter from Dr. Van Gant, be formally recorded in the claims file.

We do not seek confrontation, but our stance is evidence-based, consistent with clinical advice, and not open to dilution.

Yours sincerely,

Nicholas

Nicholas Scarr

Sent from my iPad

Louisa Scarr insurance letter (1).pdf
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